PART 1

The Emergency Approach: Safety, Triage and Assessment

IN THIS PART

1. Describe the scene and personal safety steps taken before patient contact.
2. Explain how triage assigns priority and apply a five-level acuity idea to short scenarios.
3. Describe the order of the primary survey and the check made at each step, including level of consciousness and a blood glucose check.
4. Describe how to gather a focused history and a set of vital signs, and name abnormal findings.
5. Describe a structured handover and the reassessment and documentation that go with it, within your scope of practice.

Every emergency starts the same way. Someone arrives at a scene they did not choose, takes in far more than they can process, and has a few seconds to decide what matters first. The people who handle this well are rarely the ones who know the most. They are the ones who have a fixed order of thinking and follow it while their pulse is racing.

This Part gives you that order. It begins with the ground you are standing on, because a responder who becomes a casualty helps no one. It then moves to sorting, the quiet decision about who can wait and who cannot. After that comes the primary survey, a short sequence of checks that finds the problems most likely to kill a patient soonest. The Part closes with the slower work that follows: gathering a history, measuring vital signs, handing the patient over clearly, and checking again.

Read it as a map for the rest of the book. Parts 2 to 6 deal with particular problems, but each of them assumes you can arrive safely, sort, assess in order and communicate. Where a skill depends on your training level, the text says so. What you may do is set by your scope of practice and local protocol, not by this book.

1.1 Scene Safety, Personal Protection and Responder Wellbeing

Before you kneel beside a patient, stop and look at the scene once more. Check for traffic, spilled fluid, loose electrical equipment, unstable structures, animals, and any person who may still be a danger. Your first duty is to arrive able to help. An injured responder is a second patient and takes a rescuer away from the first.

WHAT IT IS. Scene safety is the habit of judging risk before contact. It has three parts: the environment, the people present, and your own protection. A safe approach is not a delay. Taking ten deliberate seconds at the edge of a scene is faster than treating two patients instead of one.

WHY IT MATTERS. Emergencies create their own hazards. A crash scene may have traffic still moving. A patient who is confused or frightened may strike out without meaning to. A spilled fluid may be slippery or harmful to skin. Because you are focused

on the patient, you are the person least likely to notice these things, so the check must be a routine step that you do every time and not a judgment call that you make only when something looks wrong.

PERSONAL PROTECTION. Standard precautions are the baseline for every patient. They include hand hygiene, personal protective equipment, respiratory hygiene and cough etiquette, safe handling of needles and sharps, and careful cleaning and handling of equipment, textiles and the surrounding environment. The guiding rule is to use protective equipment whenever you expect possible contact with infectious material, such as blood or body fluids. For most emergency contacts that means gloves first, with eye protection, a mask or a gown added when splashes or sprays are likely. Your service, school or hospital sets the exact requirements, and you follow them.

PROCEDURE: MAKE THE SCENE SAFE

Purpose: to reach the patient without becoming a second casualty. Scope: follows your training level and local protocol.

1. Stop at the edge of the scene and look in every direction before you move in.
2. Put on gloves and any other protection your role and the situation require.
3. Name each hazard aloud, even if you are alone, so you do not skip one.
4. Remove a hazard only if it is safe and within your role; otherwise keep clear and wait for help.
5. Count the patients, and ask bystanders to keep back and, if needed, to call for more help.
6. Approach, tell the patient who you are, and begin the primary survey (Entry 1.3).

> **⚠ SAFETY**
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> Do not enter a scene you believe is unsafe. If violence, fire, unstable ground, live electrical equipment or traffic is present and you cannot control it, stay back and call for help. Giving medicines is outside many roles; follow your scope and medical direction.

STOP AND CALL FOR HELP IF a hazard remains, if you are unsure whether it has gone, or if the number of patients is more than you can manage.

The steps work because they put thinking before action. Naming hazards aloud turns a vague feeling of unease into a list you can act on, and counting patients early tells you how much help to request while you still have a free hand to request it.

RESPONDER WELLBEING. Emergency work asks you to see injury, fear and sometimes death, and it is normal for this to leave its mark. Do not treat strong reactions after a hard call as a personal failing. Eat, drink and rest when you can, tell a trusted colleague or supervisor when something stays with you, and ask whether your service or school offers a debrief or peer support. Seeking that help is part of staying safe to work, not a sign that you are unfit for it.

NURSING AND RESPONDER PRIORITIES. Check the scene before touching the patient, protect yourself with standard precautions, call for more help early if the scene or the number of patients is beyond your resources, and look after your own wellbeing as part of the job.

See also: Entry 1.3 (primary survey); Entry 1.5 (communication and handover).

1.2 Triage and Acuity

Triage is not a judgment about who deserves care. It is a way of deciding who cannot wait. A patient who walks in talking, holding a swollen ankle, can sit for a while. A patient who looks calm but is breathing far too fast cannot, and the person at the desk has perhaps thirty seconds to notice. This entry gives you a short, repeatable way to sort patients so that the quiet emergency is not missed behind the loud inconvenience.

WHAT IT IS. Triage is the process of sorting patients by urgency so that limited people, space and equipment reach the sickest first. The word comes from the French for sorting, and the idea is older than any modern emergency department. In an emergency department, triage is usually done by a nurse within minutes of arrival. At a scene with many casualties, the same logic is applied by whoever arrives first, quickly and with little equipment. Different places use different tools, and local protocol decides which one you use.

WHY IT MATTERS. Emergency departments, ambulances and scenes are never staffed for every patient at once. When demand is higher than capacity, the order in which patients are seen is itself a treatment decision. A good sorting method protects the patient who does not complain, such as the older adult with a vague story who is in fact seriously ill, from being passed over in favor of someone who is louder or more distressed but more stable.

A FIVE-LEVEL IDEA. Many emergency departments in the United States sort patients into five levels of acuity, where level one is the most urgent and level five the least. You do not need the details of any particular published tool to understand how the principle works. It rests on a small number of ordered questions.

REMEMBER

Ask four things in order: Is this patient dying now? Should this patient not wait? How much will this patient need? Are the vital signs safe for that level?

The first question asks whether the patient needs an immediate life-saving action, such as help with an obstructed airway or resuscitation. If yes, that patient is at the top level and you act at once. The second question asks whether the patient is in a high-risk situation, is newly confused or drowsy, or is in severe pain or distress. Such patients are very unwell and should not wait, even though no action needs to happen in the next few seconds. If neither applies, the remaining patients are sorted by how many different kinds of resources they are likely to need, such as laboratory tests, imaging or procedures. The last step is to check the vital signs against the level you chose and move the patient up if they are in a danger range.

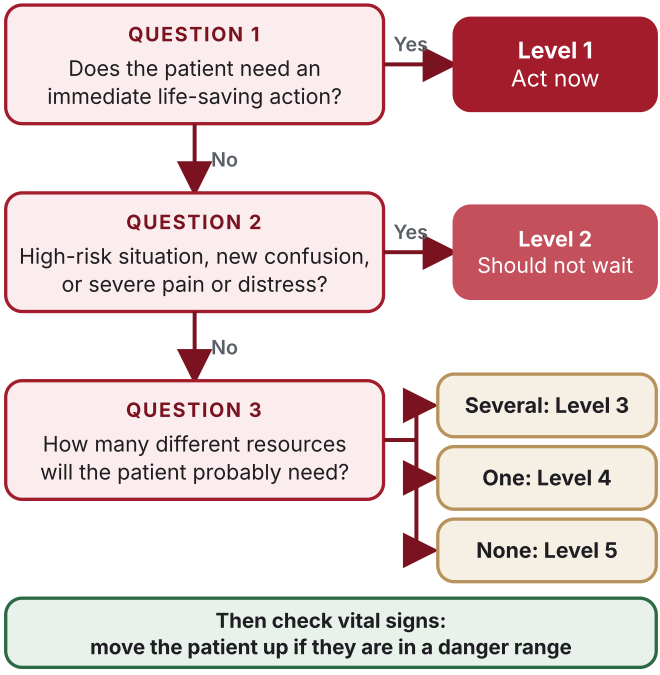


Figure 2. A generic five-level triage sequence. Notice that the first two questions are about danger, not about numbers or paperwork.

Five levels of acuity, in plain terms

Level	Plain description	Typical response
1	Needs an immediate life-saving action	Treated immediately
2	High-risk, newly confused or in severe distress	Should not wait; seen as soon as possible
3	Stable, but expected to need several resources	Waits in order, rechecked
4	Stable, expected to need one resource	Waits in order, rechecked
5	Stable, expected to need no resources	Waits in order, rechecked

A level is a starting point, not a verdict. Patients move between levels as you watch them. A person given level four on arrival who becomes short of breath in the waiting room is now level two, and the reason you recheck is to catch that change.

WHAT TRIAGE IS NOT. Triage does not diagnose. It does not say what is wrong, only how soon someone must be seen. It is also not a ranking of worth. Two patients with the same level may be seen in the order they arrived, and a patient who waits longer is not a lower-priority person, only one whose condition allows waiting. Say this to patients and families in plain words, because waiting is easier when people understand the reason for it.

COMMON PITFALLS. Four errors appear again and again. The first is anchoring on the loudest complaint and missing the quiet one. The second is trusting a calm appearance, because patients who are very ill may be quiet, tired or confused, not agitated. The third is using one set of vital signs as if it were permanent. The fourth is letting the label stick: a patient already sorted to a low

level can be forgotten for hours unless someone is responsible for rechecking. Build a habit of glancing at the waiting area each time you pass it.

RED FLAGS

Move the patient up a level and call for help for: difficulty speaking in full sentences, a gray or blue tint to the lips, new confusion or drowsiness, a patient who stops responding, severe bleeding, or a sudden change in how the patient looks compared with a few minutes ago.

SCOPE NOTE

Assigning a triage level is often a nursing role in an emergency department and is done under local protocol. Students and other responders observe or assist under supervision. If you notice a red flag in the waiting area, tell the responsible nurse at once; do not wait to be asked.

NURSING AND RESPONDER PRIORITIES. Sort by danger first and by resources second, recheck often, reassure waiting patients with an honest explanation, and escalate at once when a patient looks worse than their label.

See also: Entry 1.3 (primary survey); Entry 1.4 (vital signs); Entry 6.1 (children and acuity).

1.3 The Primary Survey

The primary survey is the short sequence of checks that finds the problems most likely to kill a patient soonest. It runs in a fixed order, and you treat a life-threatening problem the moment you find

it, before moving on. The order is Airway, Breathing, Circulation, Disability and Exposure, usually written ABCDE. This order is the spine of the whole book, and every later Part returns to it.

WHAT IT IS. The primary survey is a rapid, whole-patient look for immediate threats to life. It is not a diagnosis and it is not a full examination. Its aim is to keep the patient alive and improve their condition, which buys time for further treatment and for working out what is wrong. The same approach applies to every critically ill or deteriorating patient, whatever the cause.

WHY THE ORDER MATTERS. The letters follow how quickly each failure kills. A blocked airway stops oxygen reaching the lungs within moments, so nothing that comes later can help until the airway is open. Breathing comes next because an open airway is useless without air moving. Circulation follows because oxygen must be carried to the brain and heart. Disability, which means checking how the brain is working, comes after those, and Exposure, which means looking at the rest of the body, comes last. Severe bleeding is the one exception that rearranges the sequence. When blood loss is heavy and visible, you control it first, because there is little point in checking the airway of a patient who is bleeding to death. Entry 4.1 explains how bleeding is brought forward in trauma.

> **REMEMBER**
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> Fix each problem as you find it, then move on. Recheck after every action you take.

THE FIRST LOOK. Before the letters, take a rapid first look at the patient in general. If the patient is awake, ask a simple question such as “How are you?” If they appear unconscious, speak to them and shake the shoulders gently as you ask whether they are all right. A patient who answers in a normal voice has an open air-

way, is breathing and has enough blood flow to the brain for the moment. A patient who speaks only in short sentences may have a breathing problem. A patient who does not respond is critically ill until shown otherwise. This look takes about thirty seconds and often tells you at once that the patient needs urgent help. If the patient is unresponsive and not breathing normally, start CPR as set out in Entry 3.4. Occasional gasps are not normal breathing.

AIRWAY. Ask whether air can get in and out. Talking means the airway is open. Noisy breathing is a warning, because snoring, gurgling or a high-pitched sound means the airway is partly blocked. Complete blockage may give no sound at all, with the chest and abdomen moving in opposite directions as the patient strains. Clear and open the airway with the simple measures in Entry 2.2, and call for help at once.

BREATHING. Look, listen and feel. Count the breathing rate over a full minute if you can, and watch for effort: sweating, use of the neck and shoulder muscles, belly breathing, or a gray or blue tint to the lips. A high or rising rate is a warning that the patient may deteriorate suddenly. Note how deeply the patient breathes and whether both sides of the chest move equally. If you have a pulse oximeter, record the reading, and remember what it does not tell you, as Entry 1.4 explains.

CIRCULATION. Look at the skin of the hands: is it pink, pale, blue or mottled? Feel whether the hands are warm or cool. Check the capillary refill time by pressing on a fingertip, held level with the heart, for five seconds, then timing how long the color takes to return. In a healthy adult it is usually under two seconds, and a longer time suggests poor blood flow to the skin, though a cold room or cold hands can also lengthen it. Feel a pulse and note whether it is fast, slow, weak, strong or irregular. A bounding pulse

and a barely palpable one both matter. Look for bleeding you can see, and think about where blood might be hiding, in the chest, abdomen or pelvis. Entry 3.5 explains shock in full.

DISABILITY. This step checks how the brain is working. Begin with the quickest tool, the AVPU scale. Is the patient Alert? Do they respond only to Voice? Only to Pain? Are they Unresponsive? Many services add a letter for new confusion, so that the scale reads A, C, V, P, U, and new confusion in a previously normal patient is treated as a serious sign. Where your protocol uses it, the Glasgow Coma Scale gives a fuller picture. It scores three responses: eye opening, verbal response and motor response. The three scores are added to give a total from 3 to 15, where 15 means the patient is fully alert. Lower scores mean greater impairment, and a falling score over time matters more than a single number. Check the pupils for size, equality and reaction to light, and check the blood glucose, because low blood sugar can cause confusion or loss of consciousness and is quickly reversible. Entry 5.1 covers its causes and management.

EXPOSURE. To examine a patient properly you may need to expose the body. Do it with respect for dignity, with screens or blankets where possible, and take care that the patient does not lose heat. Look for wounds, rashes, bruising, medical alert jewelry, and anything that was hidden by clothing.

PROCEDURE: THE PRIMARY SURVEY

Purpose: to find and treat immediate threats to life in the order they kill. Scope: follows your training level and local protocol.

1. Make the scene safe and put on protection (Entry 1.1).
2. Take the first look: speak to the patient and watch the response for about thirty seconds.

3. If the patient is unresponsive and not breathing normally, call for help and begin CPR.
4. Airway: confirm the patient can speak or breathe freely; open and clear it if not.
5. Breathing: count the rate, look at the effort and chest movement, and record the oxygen saturation if a device is available.
6. Circulation: check skin color and warmth, capillary refill and pulse; control any severe bleeding.
7. Disability: check AVPU or Glasgow Coma Scale, the pupils and the blood glucose.
8. Exposure: expose as needed, protect dignity and warmth, and look for injuries.
9. Treat each life-threatening problem as you find it, call for help, and repeat the survey after every intervention.

> **⚠ SAFETY**
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> Treat the patient's cervical spine with care if there is any chance of neck injury, and do not move an injured patient unless the scene is unsafe. Wait a short while after any treatment before judging its effect, because some treatments take minutes to work. Giving medicines is outside many roles; follow your scope and medical direction.

STOP AND CALL FOR HELP IF any step finds a threat to life that you cannot correct, or if the patient does not improve after you treat it.

Each step in this procedure answers a single question, and the answers build on one another. When you finish the lettered checks, you can say whether the patient is stable, unstable, or deteriorat-

ing, and you can state in a sentence what you did and how the patient responded. That sentence becomes the core of your handover in Entry 1.5.

CHECKING BLOOD GLUCOSE. The blood glucose check is one step inside Disability, and it is a skill only where your role and protocol allow it. A meter reads a small drop of blood from a fingertip. Use the device exactly as its maker directs, wear gloves, use a new single-use lancet each time, and place the used lancet in a sharps container at once. For many people with diabetes, a reading below 70 milligrams per deciliter (3.9 millimoles per liter) is low, though the number that counts as low can differ from person to person. The number is a prompt to act under your protocol, not a diagnosis. Entry 5.1 describes the response. If your role does not include glucose testing, report that you could not check it and tell the responsible clinician.

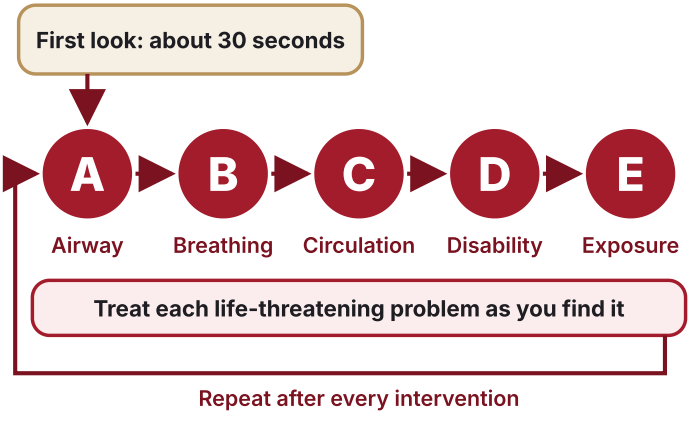


Figure 1. The primary survey as a loop. Each check is followed by treatment of any problem found, and the whole loop is repeated after every intervention.

NURSING AND RESPONDER PRIORITIES. Complete the first look quickly, work through the letters in order, treat what you find before moving on, call for help early, and repeat the sequence whenever the patient changes or after any intervention.

RED FLAGS

Unresponsiveness, absent or abnormal breathing, noisy or absent airflow, blue or gray lips, a very fast or very slow pulse, cold mottled skin, heavy bleeding, a falling level of consciousness, or a new confusion in a patient who was normal a short time ago.

See also: Entry 1.4 (vital signs); Entry 2.2 (airway); Entry 3.5 (shock); Entry 4.1 (trauma); Entry 5.1 (altered mental status).

1.4 History, Vital Signs and the Secondary Survey

Once the primary survey has found and treated immediate threats, you have time to learn more. The secondary survey is a slower, systematic look from head to toe combined with a focused history. Its job is to catch what the primary survey was not designed to find, and to give the receiving team a clear picture. It never replaces the primary survey, and you return to the primary survey whenever the patient worsens.

WHAT IT IS. The secondary survey has two strands that run together. One is the history, which asks the patient, family, bystanders and records what happened. The other is the examination, which includes a full set of vital signs and a head-to-toe check. In an unstable patient the secondary survey is shortened or postponed so that treatment and transport are not delayed.

GATHERING A HISTORY. A short, structured set of questions keeps the history focused. Ask what happened and when it started. Ask about the main symptom: where it is, how bad it is and what makes it better or worse. Ask about allergies, current medicines, past illnesses, the last meal or drink, and the events leading up to the problem. These questions are often remembered with a mnemonic, and your school or service will teach one. What matters is not the letters but the habit of covering the ground the same way every time, so that nothing important, such as an allergy or a blood-thinning medicine, is forgotten. If the patient cannot talk, turn to family, bystanders, medical alert jewelry or records, and note who gave each piece of information.

VITAL SIGNS. Vital signs are measurements of essential body functions: heart rate, breathing rate, blood pressure, temperature and oxygen saturation. Normal values change with age, sex, weight, fitness and overall health, so the ranges below describe a healthy adult at rest and should be read as a guide, not a rule.

Typical resting ranges for a healthy adult

Measure	Typical range
Pulse (heart rate)	60 to 100 beats per minute
Breathing rate	12 to 18 breaths per minute
Blood pressure	between 90/60 and 120/80 mm Hg
Temperature	97.7 to 99.1 degrees F (36.5 to 37.3 degrees C)

Treat the trend as more informative than any one reading. A breathing rate that climbs from 18 to 28 over twenty minutes tells you more than a single count, and breathing rates above about 25 breaths per minute are widely treated as a sign that a patient may

be heading for trouble. Blood pressure can stay normal for a time even in shock, because the body tightens its blood vessels to compensate, so a normal reading does not by itself make a patient safe.

OXYGEN SATURATION. A pulse oximeter clips on a finger and estimates the percentage of hemoglobin carrying oxygen. It is useful, but it has limits that you must know. It does not measure how well a patient is breathing out carbon dioxide, so a patient receiving extra oxygen can have a normal reading while still ventilating poorly. Readings can also be less accurate with poor circulation, cold hands, certain nail polish, motion and differences in skin pigmentation, and the device can be misled by some poisons. Read the number together with the patient's color, effort and mental state. A reassuring number in a patient who looks ill should not reassure you.

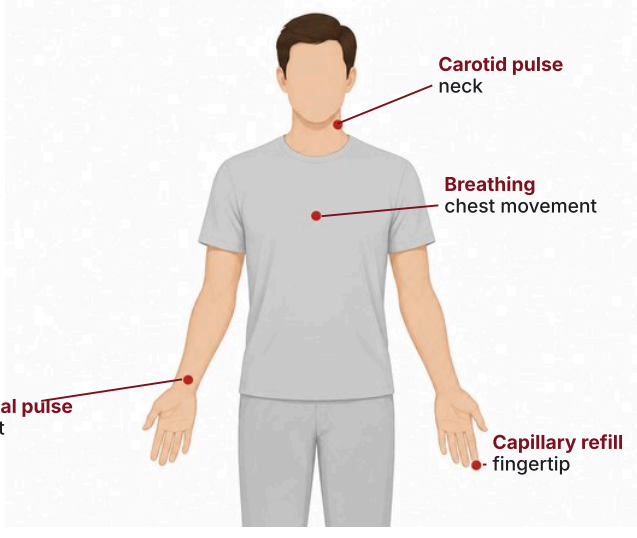


Figure 3. Where to check a pulse, breathing and skin perfusion on an adult. Check in the same places each time so that your findings can be compared.

HEAD-TO-TOE CHECK. Work from the head down: scalp and face, eyes, ears, nose and mouth, neck, chest, abdomen, pelvis, back, arms and legs. Look, then feel gently, and ask about pain. Note wounds, deformity, swelling, bruising and anything that looks wrong or smells wrong. Record the time of each set of observations. When the patient has no obvious problem, the secondary survey is your best chance to find a hidden one.

> **SCOPE NOTE**
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> Taking vital signs is a core skill for nursing, paramedic and EMT students, but interpreting patterns and acting on them is done with your supervisor and under local protocol. Always tell a qualified clinician about an abnormal or worsening finding at once.

NURSING AND RESPONDER PRIORITIES. Return to the primary survey if the patient deteriorates, take a complete set of vital signs and record the time, trust trends over single numbers, ask structured history questions the same way each time, and treat reassuring numbers with caution when the patient looks unwell.

See also: Entry 1.3 (primary survey); Entry 5.4 (sepsis); Entry 6.1 (children's values).

1.5 Communication, Consent, Handover and Reassessment

A patient cared for well by three people is easily harmed when the three do not talk to each other. This last entry covers the work that holds a response together: speaking with the patient, obtaining consent, passing information on clearly, checking again, and writing down what you saw and did.

SPEAKING WITH THE PATIENT AND CONSENT. Introduce yourself, say what you are about to do and why, and ask permission before you touch. A patient who has capacity may accept or refuse care, and you respect that choice, tell a clinician and record it. In a true emergency, when a patient cannot take part in decisions and no one who can decide for them is available, treatment may start without prior consent, and the patient is told as soon as possible. The rules differ between states, services and roles, so follow your local protocol and the guidance of your supervisor.

HANDOVER. Handover is the structured passing of a patient's information to the next team. One widely used format has four parts: Situation, Background, Assessment and Recommendation. Situation says who the patient is and what the main problem is. Background gives the history, medicines, allergies and what led up to this. Assessment gives your findings in the order of the primary survey, the vital signs and what you did. Recommendation states what you think should happen next or what you need from the receiver. A good handover is short, ordered and free of guesses presented as facts.

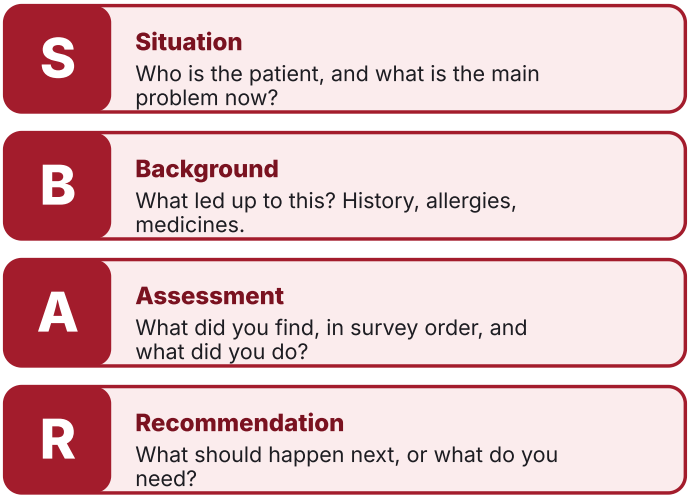


Figure 4. A four-part structured handover. Each part answers one question for the person receiving the patient.

REASSESSMENT. A patient who was stable a moment ago may not be stable now. Recheck the primary survey and vital signs at intervals set by the patient's condition and your protocol, after every treatment, and before and after any move. Wait a short while after an intervention before judging its effect.

DOCUMENTATION. Write down what you found, what you did, when you did it and how the patient responded, using the times you recorded. Record facts and observations, not opinions, and keep the record accurate and legible. A clear record protects the patient, helps the next team, and protects you.

NURSING AND RESPONDER PRIORITIES. Explain what you are doing, obtain consent or follow the emergency rules for those who cannot give it, hand over in a fixed order, recheck often, and record accurately and promptly.

See also: Entry 1.3 (primary survey); Entry 1.4 (vital signs).

Illustrative Case

ILLUSTRATIVE CASE: NOT A REAL PATIENT

Priya was a second-year nursing student volunteering at a school sports day. She was carrying a water cooler across the field when a spectator in the stands, a man in his sixties, slid sideways on the bench. People nearby stood up and shouted. Priya stopped at the foot of the stands and looked before climbing. The bleachers were crowded and the step was wet from a spilled drink, so she slowed down, pulled on gloves from her pocket kit, and asked a nearby parent to find the event first-aid lead and bring the defibrillator box.

She reached the man and said her name. His eyes opened when she spoke loudly, and he mumbled a few words. His breathing was quick and shallow, and he could say only two or three words at a time. His lips looked pale. She asked the parent to call emergency services and to give the stand number. Then she began the survey she had practiced in class.

His airway was open, because he was making sound. She counted his breathing for a full minute and found it fast. His fingertips were cool and the color took longer than usual to return after she pressed. His pulse was quick and weak. He responded to voice but not to questions, so she noted that he was not fully alert. When the first-aid lead arrived, Priya gave the handover she had rehearsed: who he was, what she had seen, her findings in order, and that he seemed to be getting worse. The lead took over and asked her to stay, record the times and keep checking his breathing until the ambulance came.

Care Priorities

CARE PRIORITIES

First, make sure you can help without being harmed. Look at the scene, protect yourself, and call for help early if the hazards or the number of patients are beyond you.

Second, find the patient who cannot wait. Sort by danger before you sort by convenience, and keep watching the waiting area, because a label given at the door can be wrong ten minutes later.

Third, assess in a fixed order and treat what you find as you find it. The first look tells you quickly whether the patient is responding normally. Then work through airway, breathing, circulation, disability and exposure, controlling severe bleeding first when it is obvious, and check again after every intervention.

Fourth, gather the history and a full set of vital signs once immediate threats are dealt with, and trust trends over single numbers.

Finally, tell the next team what you found in a fixed order, keep a clear record, and stay within your scope of practice. When something is beyond your role, saying so quickly is a safe and professional act.

Review Questions

1. What should you do first when you arrive at an emergency scene?
A. Begin the primary survey at once.
B. Check the scene for hazards and protect yourself.
C. Take a full set of vital signs.
D. Record the time on the patient's chart.

2. Triage sorts patients mainly by which factor?
A. The order in which they arrived.
B. How loudly they complain.
C. How soon they must be seen to stay safe.
D. Their expected length of stay.

3. A patient sorted to a low level of acuity becomes short of breath in the waiting room. What is the best response?
A. Leave the level unchanged, since it was assigned by a nurse.
B. Reassess, raise the level and call for help.
C. Ask the patient to wait quietly for their turn.
D. Take only their temperature.

4. In the primary survey, which letter comes after Circulation?
A. Airway.
B. Breathing.
C. Disability.
D. Exposure only if the patient is conscious.

5. A patient responds only when you speak loudly to them. On the AVPU scale, which letter describes this?
A. A.
B. V.
C. P.
D. U.

6. Which of these is a limitation of a pulse oximeter?
A. It cannot be used on a finger.
B. It always reads too high.
C. It does not show how well a patient is breathing out carbon dioxide.
D. It replaces the need to count the breathing rate.

7. In a handover that uses Situation, Background, Assessment and Recommendation, where do the primary survey findings belong?
A. Situation.
B. Background.
C. Assessment.
D. Recommendation.

8. Short case. You find an older adult slumped on a bench in a park, breathing fast and responding only to voice. List, in order, the first four things you would do.